Cancer Surveillance Assessment - Answer Key

CPH 705 - Asynchronous Evaluation (20 Points)
Author: Rauf Shah, MPH-EPI Student, COPH-UNMC

Part 1: Multiple Choice (1 point each)

Q1: b) Relative Survival Rate

Rationale: This is the specific definition of relative survival, which compares observed survival in cancer patients to expected survival in the general population matched by age, sex, and race.

Q2: c) Syndromic surveillance

Rationale: Syndromic surveillance uses provisional signals (e.g., ED chief complaints, OTC medication sales) to detect potential shifts before definitive lab or diagnosis results are available.

Q3: c) Topography Code

Rationale: The Topography code (C__._) identifies the anatomical site of the tumor. The Morphology code (M____/_) identifies cell type and behavior.

Q4: c) Eligibility Screening

Rationale: Eligibility screening follows case finding to confirm residency, diagnosis date, and reportability before full abstraction is performed.

Q5: d) Timeliness

Rationale: Timeliness measures the speed with which data move from diagnosis to being available for analysis.

Q6: c) SEER Summary Stage

Rationale: SEER Summary Stage (Localized, Regional, Distant) is a simplified system optimized for population-level analysis and public health reporting.

Part 2: Case Definition Application (8 points)

Q1: Yes

Rationale: Per Multiple Primary and Histology Rules, this presentation should be coded as a new primary.

Q2: Justification (6 points)

The 2024 presentation should be counted as a new primary because the two tumors have different ICD-O-3 morphology codes (8240 vs. 8041) and different ICD-O-3 behavior codes (/1 vs. /3). The 2021 tumor (carcinoid, behavior /1 – borderline) is non-malignant, while the 2024 tumor (small cell carcinoma, behavior /3 – malignant) is malignant. They therefore represent two distinct primary cancers under the Multiple Primary Rules, regardless of lung subsite.

Part 3: True/False (3 points each)

Q7: False

Rationale: RBAC and encryption are essential safeguards for protecting identified, confidential registry data inside the system. Public cancer registry output is typically de-identified and is protected through aggregation and small-cell suppression.

Q8: False

Rationale: Active surveillance is generally more resource-intensive and yields higher quality, verified data because staff actively seek out and confirm cases. Passive surveillance relies on facilities to report voluntarily and often has more variability in completeness and data quality.